

NHS Devon Integrated Care Board Service Specification

Service Specification No.	
Service	Diabetic injectable medication initiation
Commissioner Lead	Nicola Jones, Deputy Director for Primary Care
Provider Lead	
Period	1st October 2025 to 31 st March 2026 (3 months' notice will be required to terminate this contract by either party)
Date of Review	

1. Population Needs

1.1 National/local context and evidence base

This service specification for insulin and or injectable GLP-1 initiation in primary care outlines the details of the services to be provided. The specification of this service is designed to cover the enhanced aspects of clinical care of the patient.

This service is intended to give all practices in Devon ICB the opportunity to:

- Initiate injectable GLP1 and/or insulin in their diabetic patient populations and be remunerated. This will include dual glucose dependant insulinotropic polypeptide (GIP) and GLP-1 receptor agonists (tirzepatide).
 - Improve their confidence and competence in all areas of diabetes management
 - Develop closer working relationships with diabetes specialists
 - Simplify pathways for diabetic injectable medication initiation
 - Identify cost effective treatment options in diabetes management
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This service specification includes insulin initiation for once daily long-acting insulin or twice daily intermediate insulin.

2. Outcomes

2.1 NHS Outcomes Framework Domains & Indicators

Domain 1	<u>Preventing people from dying prematurely</u> Reducing deaths in babies and young children	✓
Domain 2	<u>Enhancing quality of life for people with long-term conditions</u> <u>Health related quality of life for people with long term conditions</u> <u>Ensuring people feel supported to manage their condition</u> <u>Improving functional ability in people with long term conditions</u>	✓
Domain 3	<u>Helping people to recover from episodes of ill-health or following injury</u> Emergency admissions for acute conditions that should not usually require hospital admission	✓

Domain 4	<u>Ensuring people have a positive experience of care</u> Patient experience of primary Care Patient experience of hospital care Friends and family Test <u>Improving children and young people's experience of healthcare</u> <u>Improving peoples experience of integrated care</u>	✓
Domain 5	<u>Treating and caring for people in safe environment and protecting them from avoidable harm</u> Patient safety Incidents reported Reducing the incidence of avoidable harm	✓

Local Defined Outcome

Patients are provided with a service in line with the locally agreed pathway and standards of care. The service delivered will be subject to one clinical audit per annum and annual review of the commissioned services in general practice, the structure of which will be developed collaboratively with the LMC.

3. Scope

3.1 Aims and objectives of service

The headline objective for this service is to ensure that no patient should have to attend hospital for insulin or injectable GLP-1 initiation with type 2 diabetes, if this can be done in primary care.

The primary aim is to ensure patients have access to safe and effective initiation of treatment for diabetes, ensuring effective interface between primary care and secondary care (from the perspective of the patient and the healthcare workforce)

In the longer term, additional benefits may accrue through:

- Improved outcomes for people with diabetes including clinical targets and care being provided locally
- Most effective use of specialist diabetes resources (e.g., reduced referrals to the outpatient department, allowing specialist services to focus on the most complex areas)
- Improved networking between primary care and the specialist team
- Greater job satisfaction for primary care clinicians

3.2 Service description/care pathway

The service description for this service section outlines:

- Initiation of insulin or GLP1 by a primary care clinician should only be done where there is sufficient competence to do so. Any other initiations should be referred for specialist input.
- The primary reason for the initiation is for treatment of type 2 diabetes and not weight management.
- The specification is for the initiation of an injectable GLP-1 or insulin in a Type 2 diabetic patient whilst adhering to the Devon Formulary and NICE guidelines (NICE NG28)
- For the purpose of this specification, tirzepatide will be included as a GLP-1 when the primary purpose is for Diabetes.

Each practice will provide a practice-based service, identifying an appropriate Clinician within the practice to provide the below

- Assessment of patient suitability for diabetic injectable medication initiation
- Educating patients and carers on self-management and self-adjustment of insulin doses
- Appropriate record keeping

- Maintain professional relations and liaison with relevant diabetes specialists
- Conduct an annual review which should include as a minimum an audit of:
 - Number of patients initiating insulin
 - Number of patients continuing on insulin at six months from initiation
 - The % of patients at different levels of IFCC HbA1c (mmol/mol)
 - Exception reporting for patients not fulfilling local guidelines for criteria for diabetic injectable medication initiation

3.3 Population covered

The registered practice population.

3.4 Any acceptance and exclusion criteria and thresholds

The service will be provided to patients who meet the following criteria:

- Type 2 diabetic
- Not achieving HbA1c targets with maximum tolerated oral combination therapy
- No other reasons for requiring hospital assessment (e.g. complex co-morbidities)
- Over 18 years
- Not pregnant
- Patient/carer is capable of safely managing their injectable medication (including, where necessary, being able to undertake home BGM, insulin injection and adjusting their own dose)
- Patient has given consent to start insulin or injectable GLP-1 and has been advised of the risks of treatment.
- There is potential that the drug could be stopped if treatment goals are not achieved.

This service specification is not for the administration of GLP-1s for weight loss therapy.

Patient Safety:

Any patient safety incidents relating to insulin or GLP-1 initiation are reported through the ICB incident reporting process.

Consent:

Practices will be required to ensure that Informed consent is sought – this may be in the form of implicit consent (where patient chooses the Service).

Prescribing:

Participating practices must ensure prescribing is in line with NICE guidance and national recommendations and should comply with the Joint Formulary.

Training and accreditation:

Each practice should ensure and provide evidence that all staff involved have adequate experience in diabetic injectable medication initiation to be sufficient to provide this service.

For detailed product information, including monitoring guidelines, please refer to the British National Formulary (BNF), the products Summary of Product Characteristics (SPC), and the formulary. Consulting these sources will ensure you have the most current and accurate information, as clinical guidance is subject to change.

3.5 Interdependence with other services/providers

Link with secondary care.

4. Applicable Service Standards

Providers and practitioners will adhere to the local and national guidelines or any updated versions thereof.

4.1 Applicable national standards (eg NICE)

The following guidance from NICE:

- Type 2 diabetes in adults: management [Overview](#) | [Type 2 diabetes in adults: management](#) | [Guidance](#) | [NICE](#)
- [Overview](#) | [Tirzepatide for treating type 2 diabetes](#) | [Guidance](#) | [NICE](#)

4.2 Applicable standards set out in Guidance and/or issued by a competent body (e.g. Royal Colleges)

Not applicable

4.3 Applicable local standards

South&West Devon Formulary and Referral: [Type 2 Diabetes Mellitus Treatment Guidance - South & West](#)

North&East Devon Formulary and Referral: [Type 2 diabetes mellitus treatment guidance - North & East](#)

Equality and Diversity:

The service must conform to legislation prohibiting discrimination and good practice in relation to equality, diversity and inclusion.

Complaints and commendations:

Participating practices are required to establish and operate a complaints and commendations procedure in line with NHS guidelines to deal with any complaints in relation to any matter connected with the provision of services. All complaints should be monitored, audited and appropriate action taken when required.

5. Applicable quality requirements and CQUIN goals

5.1 Applicable Quality Requirements (See Schedule 4 Parts [A-D])

Each practice must ensure that any personnel involved in providing any aspect of care under this scheme has the necessary training, skill and competency to do so. A Registered Nurse will supervise any care delivered by Healthcare Assistants.

5.2 Applicable CQUIN goals (See Schedule 4 Part [E])

Not applicable

6. Location of Provider Premises

The provider will provide this service from any location as included in the practice's GMS, PMS or APMS contract where these facilities enable the provider to ensure privacy and dignity and appropriate infection control.

7. Funding for 2025-2026

£229.45 per new initiation of Insulin per patient

£144.42 per new initiation of Injectable GLP-1 per patient

Nb.

- Payment is available from the commencement date of this specification for new initiations of Insulin and Injectable GLP-1.
- Payment is not available for conversions from one injectable GLP-1 to another injectable GLP-1 or from one Insulin type to another Insulin type.
- Payments are for injectable medication prescribed for diabetes treatment only (Exclusions

include oral medication or GLP-1 (including GLP-1/GIP) for Weight loss)

DDRB will be used as the start point for annual 'inflationary' review, with any deviation from DDRB uplift requiring clear rationale.

In the event of a pandemic or other 'level 4 health' event, commissioner and LMC will meet urgently to review this specification giving consideration to:

- a. suspension of some or all elements of it,
- b. redirection of patients to alternative provider(s),
- c. in the event of actioning either a) or b) determining appropriate income protection arrangements for GP provider